# Feasibility review: solo founder, Egypt first

Nov 2, 2026

**Verdict: Go, as a small phone-only MVP, but only after a 30-day validation passes.** One person can build and run this in Egypt for under about $1,500 in year one (plus their time). It needs no servers, and phone-only avoids most data-protection licensing. The idea is well supported by desk research. Whether families will **actually switch** from keeping prescriptions in their WhatsApp chats and photo gallery is not yet proven.

**Conditions for the Go**

1. Validate first: interviews, a waitlist, a prototype test, and a hand-run concierge trial (Validation section).
2. Keep v1 phone-only. No cloud, no partner portal, no ads, no clinical calculators.
3. Ship Android and iOS from one Flutter codebase, but test mainly on budget Android phones: Android is 82% of Egyptian mobile use.
4. Get one hour of Egyptian legal advice on the privacy policy and the data-protection law (PDPL) before launch, and full review before any cloud feature.

## Solo-founder MVP scope

The proposal describes about 40 features; a solo v1 should ship roughly a quarter of them. The test for each one: **does it work with no server, and does it help a family on day one?**

| Build in v1 (phone only) | Move to v1.1 or later | Why later |
|---|---|---|
| Family profiles (no logins), timeline per person | Family sync between phones, guardian accounts | Needs a server, accounts and the PDPL licence |
| Visits, doctor cards, medicines, scans, labs, vaccines, surgeries, with photos | Caregiver missed-dose alerts to another phone | Needs a server + push |
| Reminders and full-screen alarms, "Taken/Snooze", home-screen widget | Cloud OCR for handwriting | Data transfer + cost; on-device first |
| On-device OCR + barcode for medicine boxes | Partner portal, clinic reminders, sponsored listings | Needs users first |
| Insurance cards + WhatsApp sharing | Health Connect / Apple Health import | Extra store permissions and reviews |

| Build in v1 (phone only) | Move to v1.1 or later | Why later |
|---|---|---|
| Kids: vaccine schedule + growth chart (shown as information) | Growth flags, dose calculator, interaction warnings | Possible medical-device rules |
| Spending dashboard | Pregnancy Mode | Valuable; v1.1 once the core is stable |
| Emergency card + SOS call | SMS/WhatsApp reminders for feature phones | Paid per message |
| Simple Mode for elderly, Arabic + English | Lab trends, visit voice notes, multi-device | Polish after usage data |
| Encrypted backup file (save to Google Drive or share) + restore | Ads, analytics beyond crash reports | Trust first |

**Build effort (estimate)**: about 3–5 months full-time for one experienced developer using Flutter, a local database and on-device ML Kit. Most of the time goes into reliable reminders on many Android brands, Arabic right-to-left layouts, and backup/restore. Design can start from the screen prototype.

## Year-one costs for one person

| Item | Cost (estimate) | Notes |
|---|---|---|
| Apple Developer Program | $99 / year | Required for iOS |
| Google Play developer account | $25 once | |
| Domain + simple landing page | ~$15–50 / year | Arabic waitlist page for validation |
| Firebase Crashlytics, push | $0 | Free tiers |
| iOS builds without owning a Mac | $0 on a CI free tier (e.g. Codemagic), or a used Mac | |
| Test phones: one budget Android + one used iPhone | ~$300–600 once | Test on the phones families actually use |
| Validation ads (Facebook / Instagram) | ~$100–300 | For the waitlist test |

| Item | Cost (estimate) | Notes |
|---|---|---|
| Egyptian lawyer: privacy policy + PDPL check | Get 2–3 quotes | Do not skip |
| **Total, excluding the lawyer and your time** | **~$550–1,100** | No servers in v1 |

Costs that start only with cloud features (v2): in-country hosting, a PDPC licence and a data protection officer, and penetration testing. Plan to fund those from partner revenue or a grant, not before.

## Egypt market check

The need in Egypt is strong and well documented. The biggest competitor is not another app: it is **the WhatsApp chat and the phone's photo gallery** where families already dump prescriptions and reports. v1 must be clearly easier than that.

| Strengths | Evidence |
|---|---|
| Families pay most costs themselves | 57.2% of health spending out of pocket, 2023 (World Bank) |
| Almost everyone is online | 98.2M internet users (82.7%), Oct 2025 (DataReportal) |
| Android-heavy market | Android 82.1%, iOS 17.9% of mobile use, Oct 2026 (StatCounter) |
| Many children and chronic patients | 2.74 births per woman; 22.4% of adults with diabetes (World Bank) |
| No direct competitor | Vezeeta, Yodawy, Chefaa, Altibbi don't keep family history + spending; universal insurance still rolling out by governorate, no patient app found |

| Risks | What to do about it |
|---|---|
| Habit: "I just keep the photos on WhatsApp" | Make the first 60 seconds magic: snap a prescription → medicine + reminder created automatically |
| Low trust in apps holding medical data | "Your data never leaves your phone" as the headline; no login needed in v1 |

| Risks | What to do about it |
|---|---|
| Older users with low digital literacy | Simple Mode, voice, pill photos; the adult child sets it up |
| Budget phones: little storage, aggressive battery savers | Compress photos to ~200 KB; guided battery-settings setup; test on cheap Xiaomi/Oppo/Samsung |
| Free app, unproven revenue | Keep costs near zero until clinic partnerships are tested in v2 |
| PDPL enforcement from late 2026 | Phone-only v1 keeps you out of most licensing; legal check before any cloud |

## Phone-first now, cloud later

**Yes, this works, if v1 is built "sync-ready" from day one.** Then adding a cloud option later is a feature, not a rewrite.

### Build these into v1 now (no server needed)

- Every record gets a permanent unique ID, a "last changed" time, and soft delete (marked deleted, not erased). That is what sync needs later.
- Photos stored separately from the records, compressed, each with its own ID.
- One encryption key per family, kept in the phone's secure keystore (Android Keystore / iOS Keychain). Backups are encrypted with it.
- A documented export format: an encrypted archive of records (JSON) + photos, restorable on any phone.
- A recovery key the user writes down or saves, so a lost phone does not mean lost history.

### Cloud options, in the order to add them

| Step | Option | What it gives | Server cost | Legal weight (Egypt) |
|---|---|---|---|---|
| v1 | Encrypted backup file saved to the user's own Google Drive (or shared anywhere) | Safety against a lost phone | $0 | Lowest: data encrypted, user-initiated. Confirm with a lawyer whether it counts as a transfer abroad |

| Step | Option | What it gives | Server cost | Legal weight (Egypt) |
|---|---|---|---|---|
| v2 | **End-to-end encrypted** backup and sync on your own server: the server stores data it cannot read | Family sync, caregiver alerts, new-phone restore | Low at first | PDPC licence, DPO; host **in Egypt**. Huawei Cloud has a Cairo region (May 2024); no AWS, Google or Azure region in Egypt was found |
| v3 | Readable cloud data (for partner records, cloud OCR) | Clinic integrations | Higher | Full compliance: licence, DPO, impact assessment, contracts with each clinic |

Not iCloud: Apple forbids storing personal health information there (guideline 5.1.3).

## Validation

**What is supported by research:** the problem exists (high out-of-pocket costs, paper records, many children and chronic patients) and no app solves it in Egypt. **What is not proven yet:** that families will switch from WhatsApp/gallery, keep entering data after week one, and that clinics will later pay. Those three need real people, not desk research.

**30-day validation plan**

- [ ] **Week 1: 15–20 interviews.** 5 mothers of children under 5, 5 adult children caring for elderly parents, 5 people with a chronic condition, 3–5 clinic receptionists or doctors. Ask about the last time a paper got lost, a dose was missed, or a cost surprised them. Don't pitch the app.
- [ ] **Week 2: Arabic landing page + waitlist.** Post in Egyptian parenting and caregiver Facebook groups; ~$100 of ads. Measure sign-ups.
- [ ] **Week 3: clickable prototype test** with 8–10 people, including 3 aged 60+. Task: "Add Mum's new medicine and set the reminder."
- [ ] **Week 4: concierge trial.** 10 families send prescription photos to a WhatsApp number; you enter them and send reminders by hand. This tests real behaviour before writing code.

**Go / no-go criteria** (suggested targets; adjust after week 1)

| Test | Go if | If it fails |
|---|---|---|
| Interviews | 60%+ describe a recent lost paper, missed dose or cost confusion | Problem is weaker than thought: narrow the target group |
| Waitlist | 300+ sign-ups in 2 weeks | Sharpen the message; test "medicine reminders for parents" vs "kids' vaccines" |
| Prototype | 7 of 10 finish the task alone, including 2 of the 3 older testers | Simplify the flows before building |
| Concierge trial | 5 of 10 families still sending records in week 4 | Habit problem: rethink the entry point (e.g. WhatsApp-first capture) |
| Clinics | 2+ say they'd pilot appointment reminders | Revenue path unproven: keep costs minimal, look for grants |

**If it fails, pivot to a narrower wedge**: medicine reminders for elderly parents (managed by their adult children), or children's vaccines + growth only. Both are subsets of the same app.

## Risks and mitigations

| Risk | Likelihood | Impact | Mitigation |
|---|---|---|---|
| Users don't keep entering data after week one | High | High | Snap-to-record OCR, auto-created reminders, weekly "family health digest" notification |
| Reminders silently killed on budget Android phones | High | High | Exact-alarm permission, battery-settings guide per brand, test on 3+ cheap brands |
| Lost phone = lost history | Medium | High | Encrypted backup + recovery key in v1; nudge to back up monthly |
| Solo founder burnout / scope creep | High | High | Ship the v1 scope table only; everything else waits for validated demand |

| Risk | Likelihood | Impact | Mitigation |
|---|---|---|---|
| Legal: PDPL licence or medical-device rules | Medium | High | Phone-only v1; keep calculators and flags out; lawyer before cloud |
| Store rejection | Low | Medium | Privacy label, Data safety form, health declaration, "not a medical device" disclaimer, in-app deletion |
| No revenue | Medium | Medium | Near-zero costs; clinic reminders + sponsored listings tested in v2 |
| A big player (Vezeeta, Sahatna-like gov app) adds family records | Medium | Medium | Win on offline, privacy, elderly care and costs: things booking apps don't focus on |

*Likelihood and impact are judgement calls; targets and costs are estimates. Sources are in the MENA research tab and linked inline.*